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ATTENTION:
Before working in this template, choose File, Save As... to an appropriate folder.
Modify the file name according to company procedures.

PROTOCOL (PHASES 2 AND 3)

CONTENT TEMPLATE

For use with DXC's

Writer™ Master Template

Use this Writer Content Template to write a Phase 2 or 3 clinical trial protocol. The format of this template is compliant with International Council for Harmonization (ICH) Guidelines for the applicable Module 5 folder of the electronic Common Technical Document (eCTD).

Instructional text source: ICH E6 (R2) Guideline for Good Clinical Practice (Section 6).

DO NOT MODIFY OR DELETE ANY STYLES IN THIS TEMPLATE.

CLINICAL TRIAL PROTOCOL: {STUDY NUMBER}

The fields on this Title Page and in the headers are populated using options from the Writer toolbar.

ALL information in the fields enclosed in curly brackets should be entered using the Document Information dialog box. Entering information directly into the fields on the Title Page or in the header may invalidate existing bookmarks connecting fields to the header and/or other locations in this document.

Use the Help button on the Writer toolbar or refer to the Writer User Manual before modifying the fields in this template.

Document title pages can be easily created using a tabular format for an orderly, consistent appearance. The following table is provided as a sample; it does not represent suggested contents. Modify, delete, or add rows to the table as needed. The page header may be modified also, eg, by replacing the "final date" field with the "revision date" field.

Study Title:

Study Number:

Study Phase:

Product Name:

IND Number:

Indication:

Investigators:

Sponsor:

Sponsor Contact:

Sponsor's Legal Representative:

Medical Monitor:

Version:

Version Date:

Confidentiality Statement

This document contains confidential and proprietary information of {sponsor} that must not be disclosed to anyone other than the recipient, study staff, and members of the Institutional Review Board/Independent Ethics Committee. This information cannot be used for any other purpose other than the evaluation or conduct of the clinical study without the prior written consent of {sponsor}. Include a statement instructing the Investigator/recipients to consider this protocol a confidential document and to treat it as such.

GENERAL NOTES:

Determine whether to use the term “subject,” “participant,” or “patient” in the protocol and then use that term consistently throughout the protocol. “Subject” is used in this protocol. Change if applicable.

SYNOPSIS

SYNOPSIS INSTRUCTIONS:

Information in the Synopsis should be accurate and consistent with text and data provided in the document body. The Synopsis must be updated with each new document version and dated accordingly.
The Synopsis should be brief (1 to 3 pages) and should summarize the document content.

Sponsor:
{sponsor}

Name of Finished Product:
{product name}

Name of Active Ingredient:

Study Title:
{study title}

Study Number:
{study number}

Study Phase: {study phase}

Study Rationale: *Add a brief description of the study rationale here as described in the body.*

Primary Objective(s) and Endpoint(s): Align the endpoints with their respective objectives.

Objective(s)	**Endpoint(s)**

Secondary Objective(s) and Endpoint(s):

Objective(s)	**Endpoint(s)**

Exploratory Objective(s) and Endpoint(s)

Objective(s)	Endpoint(s)

Study Design:

Study Population:
subjects are planned.

Diagnosis and Criteria for Inclusion:
Copy all inclusion and exclusion criteria

Test Product, Dose, and Mode of Administration:

Reference Product, Dose, and Mode of Administration:

Duration of Treatment:

Efficacy Assessments:

Safety Assessments:

Statistical Methods:

Version Number: {version number}

Version Date: {version date}

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LIST OF ABBREVIATIONS AND DEFINITIONS OF TERMS

Provide a list of the abbreviations and definitions of specialized or unusual terms or measurement units used in the protocol. Listing standard abbreviations, eg, units of measurement such as mg or cm is not necessary.

Abbreviated terms should be spelled out and the abbreviation indicated in parentheses at first appearance in both the synopsis and the body of the protocol.

A list of commonly used abbreviations appears below. Delete those abbreviations that are not used in this protocol. Add additional abbreviations as necessary.

This section may be divided into 2 subsections if applicable: List of Abbreviations and Definitions of Terms. Use the style Heading 2 Unnumbered to define these sections.

If no terms are defined in this section, the words “And Definitions of Terms” may be deleted from the above heading to read “List of Abbreviations.”

AE	adverse event
AESI	adverse event of special interest
ALB	Albumin
ALK-P	alkaline phosphatase
ALT	alanine aminotransferase
ANCOVA	analysis of covariance
ANOVA	analysis of variance
aPTT	activated partial thromboplastin time
AST	aspartate aminotransferase
bid	twice daily
BMI	body mass index
BUN	blood urea nitrogen
CFR	Code of Federal Regulations
CI	confidence interval
CLIA	Clinical Laboratory Improvement Amendments
CMH	Cochran-Mantel-Haenszel
CMP	Clinical Monitoring Plan
CONSORT	Consolidated Standards of Reporting Trials

COSTART	Coding Symbols for a Thesaurus of Adverse Reaction Terms
CRF	case report form
CTCAE	Common Terminology Criteria for Adverse Events
CTFG	Clinical Trials Facilitation Group
CV	coefficient of variation
DCC	Data Coordinating Center
DRE	disease-related event
DSMB	Data and Safety Monitoring Board
ECG	Electrocardiogram
eCRF	electronic case report form
EDC	electronic data capture
FDA	Food and Drug Administration
FSH	follicle-stimulating hormone
GCP	Good Clinical Practice
GGT	gamma-glutamyltransferase
GLP	Good Laboratory Practice
GMP	Good Manufacturing Practice
HBsAg	hepatitis B surface antigen
hCG	human chorionic gonadotropin
Hct	Hematocrit
HCV	hepatitis C virus
Hgb	Hemoglobin
HIPAA	Health Information Portability and Accountability Act
HIV	human immunodeficiency virus
HRT	hormone replacement therapy
IB	Investigator's Brochure
IC	Institute or Center
ICF	informed consent form
ICH	International Council for Harmonization

IDE	Investigational Device Exemption
IEC	Independent Ethics Committee
IND	Investigational New Drug
IP	investigational product
IRB	Institutional Review Board
ISM	Independent Safety Monitor
ITT	intent-to-treat
IUD	intrauterine device
IUS	intrauterine hormone-releasing system
LAM	lactational amenorrhea method
LDH	lactate dehydrogenase
MCH	mean corpuscular hemoglobin
MCHC	mean corpuscular hemoglobin concentration
MCV	mean corpuscular volume
MedDRA	Medical Dictionary for Regulatory Activities
MOP	Manual of Procedures
MSDS	Material Safety Data Sheet
NDA	New Drug Application
NIH	National Institutes of Health
OHRP	Office for Human Research Protections
PI	Principal Investigator
PT	prothrombin time
PV	pharmacovigilance
QC	quality control
qd	once daily
qid	four times daily
RBC	red blood cell (count)
SAE	serious adverse event
SD	standard deviation

SE	standard error
SMC	Safety Monitoring Committee
SOC	system organ class
SOE	schedule of events
SOP	standard operating procedure
SUSAR	suspected unexpected serious adverse reaction
tid	three times daily
US	United States
WBC	white blood cell (count)
WOCBP	woman of childbearing potential

1 INTRODUCTION

The Protocol Introduction should not exceed 1 to 2 pages in length and should include the following information:

1.1 Study Rationale

State the problem or question (eg, describe the population, disease, current standard of care, if one exists, and limitations of knowledge or available therapy) and the reason for conducting the clinical trial.

1.2 Background

This section should include:

- A summary of findings from nonclinical in vitro or in vivo studies that have potential clinical significance
- A summary of relevant clinical research and any history of human use or exposure to the study intervention, including use in other countries, and clinical pharmacology studies
- Discussion of important literature and data that are relevant to the trial and that provide background for the trial (reference citations should be listed in Section 11, References)
- Applicable clinical, epidemiological, or public health background or context of the clinical trial
- Importance of the clinical trial and any relevant treatment issues or controversies

1.3 Risk Benefit Assessment

1.3.1 Known Potential Risks

Include a discussion of known potential risks from either clinical or nonclinical studies. If a package insert or device labeling from a licensed or approved product is available, it should be used as the primary source of risk information. If the product is investigational, the Investigator's Brochure (IB) should be the primary source of the risk information. In addition, relevant published literature can also provide relevant risk information. If the risk profile cannot be described from the package insert, device labeling, or the IB, the risk information discussion will result from published literature and should be included and referenced appropriately.

Describe any physical, psychological, social, legal, economic, or any other risks to subjects by participating in the study that the Principal Investigator (PI) foresees, addressing each of the following:

- Immediate risks

- Long-range risks

  If risk is related to proposed procedures included in the protocol, describe alternative procedures that have been considered and explain why alternative procedures are not included

1.3.2 Known Potential Benefits

Include a discussion of known potential benefits from either clinical or nonclinical studies. If a package insert or device labeling from a licensed or approved product is available, it should be used as the primary source of potential benefit information. If the product is investigational, the IB should be the primary source of the potential benefit information. In addition, relevant published literature can also provide potential relevant benefit information. If the potential benefit cannot be described from the package insert, device labeling, or the IB, the potential benefit information discussion will result from published literature and should be included and referenced appropriately.

Describe any physical, psychological, social, legal, or any other potential benefits to individual subjects or society in general, as a result of participating in the study, addressing each of the following:

- Immediate potential benefits
- Long-range potential benefits

Note that payment to subjects, whether as an inducement to participate or as compensation for time and inconvenience, is not considered a "benefit." Provision of incidental care is also not to be considered a benefit.

1.3.3 Assessment of Potential Risks and Benefits

Include an assessment of known potential risks and benefits, addressing each of the following:

- Rationale for the necessity of exposing subjects to risks and a summary of the ways that risks to subjects were minimized in the study design
- Justification as to why the risks of participation in the study outweigh the value of the information to be gained

1.4 Mechanism of Action

Include the investigational product (IP)'s mechanism of action if known.

2 STUDY OBJECTIVES AND ENDPOINTS

Provide a description of the study objectives and endpoints, as well as a justification for selecting the particular endpoints, in the table format included below. This will provide clear articulation of how the selected primary and secondary endpoint(s) are linked to achieving the primary and secondary objectives. Data points collected in the study should support an objective or have a regulatory purpose. Therefore, careful consideration should be given prospectively to the amount of data needed to support the study's objectives.

An objective is the purpose for performing the study in terms of the scientific question to be answered. Express each objective as a statement of purpose (eg, to assess, to determine, to compare, to evaluate) and include the general purpose (eg, efficacy, effectiveness, safety) and/or specific purpose (eg, dose-response, superiority to placebo, effect of an intervention on disease incidence, disease severity, or health behavior).

A study endpoint is a specific measurement or observation to assess the effect of the study variable (study intervention). Study endpoints should be prioritized and should correspond to the study objectives and hypotheses being tested. Give succinct, but precise definitions of the study endpoints used to address the study's primary objective and secondary objectives (eg, specific laboratory tests that define safety or efficacy, clinical assessments of disease status, assessments of psychological characteristics, subject-reported outcomes, behaviors or health outcomes). Include the study visits or time points at which data will be recorded or samples will be obtained. Describe how endpoint(s) will be adjudicated, if applicable.

Primary and secondary endpoints should be adjusted for multiplicity. If a claim is sought for the secondary endpoints, the statistical plan for adjustment for multiplicity should be aligned with those objectives.

Study objectives should provide an answer to the primary question(s) to be addressed in the trial.

In Phase 2-3 trials, objectives could include (but are not limited to):

- Evaluating the efficacy of the IP in treating the indicated disease.
- Evaluating therapeutic dose levels and dosage frequency.
- Evaluating the safety and tolerability of the IP.
- Determining the risks and benefits of short- or long-term therapy.

2.1 Primary Objective(s) and Endpoint(s)

Insert the objectives and endpoints together in this section in tabular format as applicable to the study in the following style: "To evaluate…"

Objective(s)	**Endpoint(s)**

2.2 Secondary Objective(s) and Endpoint(s)

Objective(s)	Endpoint(s)

2.3 Exploratory Objective(s) and Endpoint(s)

Objective(s)	Endpoint(s)

3 INVESTIGATIONAL PLAN

3.1 Overall Study Design and Plan

The scientific integrity of the trial and the credibility of the data from the trial depend substantially on the trial design.

Provide a brief overall description of the trial design, similar to a short abstract. Details of each item described in this section will follow in other sections of this protocol template. Include descriptions of the following, as applicable:

- A statement of the hypothesis
- Phase of the trial
- A description of the type/design of trial to be conducted (eg, randomized, placebo-controlled, double-blinded, parallel design, open-label, dose escalation, dose-ranging, adaptive, cluster randomized, group sequential, multi-regional, superiority or non-inferiority design)
- A description of methods to be used to minimize bias
- Dose escalation or dose-ranging details should be contained in Section 5, Study Treatments
- The number of study groups/arms and study intervention duration
- Indicate if single site or multi-site
- Name of study intervention(s)
- Note if interim analysis is planned and refer to details in Section 9, Planned Statistical Methods
- Note if the study includes any stratifications and if so, identify the stratification planned (eg, sex, race/ethnicity, age, dose) and refer to details in Section 9, Planned Statistical Methods
- Name of sub-studies, if any, included in this protocol

If other studies used a very similar protocol, consider discussing any important differences between the protocols.

Use charts and diagrams as needed. Include an events table that includes timing of assessments in an appendix to the protocol. Be certain that the events table exactly matches the methods and procedures described in the protocol text.

See Appendix 1, Schedule of Events (SOE).

3.2 Rationale for Study Design and Control Group

- Provide a rationale for the selected study design and comparator group(s) chosen.
- Provide a rationale for the doses or dose ranges to be used in the study for all treatments, and describe the basis for choosing them (eg, prior experience in humans, animal data).

- In addition to the type of control, other critical design features that may need discussion are the use of a cross-over design and the selection of subjects with particular prior history, such as response or non-response to a specific drug or member of a drug class.
- Consider the following questions:
  - What are the primary questions that the study will be designed to answer?
  - Will the study design support the intended labeling?
  - What is the rationale for the dose that was chosen?
  - Why was the design chosen?
  - What were the designs that were rejected, and why?
  - What are the potential benefits of the test product over the comparator or standard treatment?

3.3 Justification for Dose

Provide a justification for the route of administration, planned maximum dosage, and dosing regimen, including starting dose, of the study intervention(s) and control product(s).

3.4 Study Duration

- Describe the expected duration of the study, with proposed subject recruitment start date and end date.
- Include a statement that the actual overall study duration or subject recruitment period may vary.
- Explain the justification for the overall duration of the study.

A clinical trial is considered completed when subjects are no longer being examined or the last subject's last study visit has occurred.

Example text provided as a guide, customize as needed:

[A subject is considered to have completed the study if he or she has completed all phases of the study including the last visit or the last scheduled procedure shown in the SOE in Appendix 1.

The end of the study is defined as completion of the last visit or procedure shown in the SOE in the trial globally.]

4 STUDY POPULATION SELECTION

Use the following guidelines when developing subject eligibility criteria to be listed in Sections 4.1 Inclusion Criteria and 4.2 Exclusion Criteria:

- The eligibility criteria should provide a definition of subject characteristics required for study entry/enrollment.
- If subjects require screening, distinguish between screening subjects vs enrolling subjects. Determine if screening procedures will be performed under a separate screening consent form.
- The risks of the study intervention should be considered in the development of the inclusion/exclusion criteria so that risks are minimized.
- The same criterion should not be listed as both an inclusion and exclusion criterion (eg, do not state age >18 years old as an inclusion criterion and age ≤ 18 years old as an exclusion criterion).
- Identify specific laboratory tests or clinical characteristics that will be used as criteria for enrollment or exclusion.
- If reproductive status (eg, pregnancy, lactation, reproductive potential) is an eligibility criterion, provide specific contraception requirements (eg, licensed hormonal or barrier methods).
- If you have more than one study population, please define the common inclusion and exclusion criteria followed by the specific inclusion and exclusion criteria for each subpopulation.

4.1 Inclusion Criteria

- Insert the list of inclusion criteria.
- Use a numbered list for the inclusion criteria for easier identification of an individual criterion if any amendments are needed.

- Inclusion criteria should be single, stand-alone criteria ideally that can be answered yes or no.

Some suggested text follows:

Each subject must meet the following criteria to be enrolled in this study.

1.

2.

3. Refer to Appendix 5

4.

5.

6.

7.

8.

4.2 Exclusion Criteria

- Insert the list of exclusion criteria.
- Use a numbered list for the exclusion criteria for easier identification of an individual criterion if any amendments are needed.
- Exclusion criteria should be single, stand-alone criteria ideally that can be answered yes or no.

Some suggested text follows:

Subjects who meet any of the following criteria will be excluded from the study.

1.

9. Refer to Appendix 5

10.

11.

12.

13.

14.

4.3 Screen Failures

Subjects who are consented to participate in the clinical trial, who do not meet one or more criteria required for participation in the trial during the screening procedures, are considered screen failures. Indicate how screen failures will be handled in the trial, including conditions and criteria upon which re-screening is acceptable, when applicable.
Example text provided as a guide, customize as needed:

[Screen failures are defined as subjects who consent to participate in the clinical trial but are not subsequently randomly assigned to the study intervention or entered in the study. A minimal set of screen failure information is required to ensure transparent reporting of screen failure subjects, to meet the Consolidated Standards of Reporting Trials (CONSORT) publishing requirements and to respond to queries from regulatory authorities. Minimal information includes demography, screen failure details, eligibility criteria, and any serious adverse event (SAE).

Individuals who do not meet the criteria for participation in this trial (screen failure) because of a may be rescreened. Rescreened subjects should be assigned the same subject number as for the initial screening.]

4.4 Strategies for Recruitment and Retention

This section may be deleted in favor of having this information in the MOP, especially if recruitment strategies have not been finalized.*Identify general strategies for subject recruitment and retention. This section may refer to a separate detailed recruitment and retention plan in the manual of procedures (MOP) and site-specific plans could be included in a site-specific standard operating procedure (SOP). Consider inclusion of the information below either in this section or the MOP.*

- Target study sample size by gender, race and ethnicity, and age; identify anticipated number to be screened including women and minorities in order to reach the target enrollment (should be consistent with information contained in Section 9.2, Determination of Sample Size).
- Anticipated accrual rate.
- Anticipated number of sites and subjects to be enrolled from the US and outside the US.
- Source of subjects (eg, inpatient hospital setting, outpatient clinics, student health service, or general public).
- Recruitment venues.
- How potential subjects will be identified and approached.
- Types of recruitment strategies planned (eg subject advocacy groups, national newspaper, local flyers; social media, specific names of where advertisements may be planned are not needed).
- If the study requires long-term participation, describe procedures that will be used to enhance subject retention (eg, multiple methods for contacting subjects, visit reminders, incentives for visit attendance).
- If applicable, specific strategies that will be used to recruit and retain historically under-represented populations in order to meet target sample size and conform with the NIH Policy on Inclusion of Women and Minorities as Subjects In Research Involving Human Subjects. Include the number of women and minorities expected to be recruited, or provide justification on those rare occasions where women and/or minorities will not be recruited.

In a*ddition, this section should address:*

- If appropriate, include justification for inclusion of vulnerable subjects and recruitment strategy. Vulnerable subjects include, but are not limited to pregnant women, those who lack consent capacity, including the mentally ill, prisoners, cognitively impaired subjects, children, and employee volunteers. Include safeguards for protecting vulnerable populations. Please refer to OHRP guidelines when choosing the study population. Note that these regulations apply if any subjects are members of the designated population, even if it is not the target population (eg, if a subject becomes a prisoner during the study).

- If subjects will be compensated or provided any incentives (eg vouchers, gift cards,) for study participation, describe amount, form and timing of such compensation in relation to study activities (include financial and non-financial incentives). Describe who will receive incentives (if not the subject). For example, if minors, state whether the minor or the parent/guardian will receive the incentive. If incapacitated adults, state if payment will be provided to the subject or to a legally authorized representative or guardian.

5 STUDY TREATMENT(S)

5.1 Description of Treatment(s)

Describe the study intervention(s) and control product. Product information can usually be obtained from the:

- IB for an investigational drug or biologic
- Package insert for a licensed or approved drug or biologic or device labeling for a licensed device
- Proposed labeling and/or material safety data sheet (MSDS) for an investigational device
- Final labeling for a marketed device

5.1.1 Study Drug

5.1.2 Placebo or Control

5.2 Treatments Administered

Provide the following information:

- The trial treatment(s) to be administered, including the trade and generic names(s) of all the product(s), as well as the name(s) of active and inactive ingredients.
- The dose(s).
- The route/mode(s) of administration (eg, oral, nasal, intramuscular).
- The dosing schedule.
- The treatment period(s), including the follow-up period(s) for subjects for each IP treatment/trial treatment group/arm of the trial.
- The type of comparator(s) (eg, placebo, no treatment, active drug, dose-response) and study configuration (parallel, cross-over).
- The method for dispensing study treatment to the subject and who will be responsible for dispensing study treatment.

The following figure is provided as a sample of the template figure format.

Figure 1. Sample Figure

5.3 Selection and Timing of Dose for Each Subject

Provide the following information:

- Procedures for selecting each subject's dose of test drug/IP and active control/comparator. These procedures can vary from simple random assignment to a selected fixed drug/dose regimen, to some specified titration procedure, to more elaborate response-determined selection procedures, eg, where dose is titrated upward at intervals until intolerance or some specified endpoint is achieved.
- Procedures for back-titration, if any.
- The timing (time of day, interval) of dosing and the relation of dosing to meals. If appropriate, state that there is no relationship to the timing of meals.
- Any specific instructions to be given to subjects about when or how to take the dose(s).

5.4 Dose Escalation/Modification

If applicable, describe the dose escalation scheme and dose regimen (using exact dose, rather than percentages). State any minimum period required before a subject's dose might be raised to the next higher dose or dose range. If applicable, the protocol should state the conditions under which a dose change will be made, particularly in regard to failure to respond or to toxic or untoward changes in stipulated indicators (eg, white blood cell count in cancer chemotherapy).

Address dose modifications for specific abnormal laboratory values of concern or other adverse events (AEs) that are known to be associated with the planned study intervention. The protocol must state explicitly the dose-limiting effects that are anticipated. Provide criteria that will be used to determine dose escalations. If a subject is responding positively to the intervention, the protocol should specify whether study intervention administration would progress to still higher doses. If appropriate, provide a dose de-escalation schema with intervention modifications. Do not restate reasons for withdrawal of subjects. Cross-reference relevant sections, as appropriate.

5.5 Missed Dose

Include any specific instructions about how delayed or missed doses should be handled. Include any specific instructions or safety precautions for administration of the study intervention.

5.6 Randomization and Blinding/Masking

This section should contain a description of randomization and blinding (or masking for vision studies) procedures (if applicable to the study design). It should include a description or a table that describes how study subjects will be assigned to study groups, without being so specific that blinding or randomization might be compromised (eg, the ratio between intervention and placebo groups may be stated). If adaptive randomization or other methods of covariate balancing/minimization are employed, include a cross link to the methods of analysis in Section 9, Planned Statistical Methods. In addition, details regarding the implementation of procedures to minimize bias should be included in this section. DO NOT include details that might compromise these strategies. Design techniques to avoid bias can be found in the ICH Guidance for Industry E9 Statistical Principles for Clinical Trials.

Plans for the maintenance of trial randomization codes and appropriate blinding for the study should be discussed. The timing and procedures for planned and unplanned breaking of randomization codes should be included. Include a statement regarding when unblinding may occur and who may unblind. Provide the criteria for breaking the study blind or subject code. Discuss the circumstances in which the blind would be broken for an individual or for all subjects (eg, for SAEs). Indicate to whom the intentional and unintentional breaking of the blind should be reported.

Sometimes blinding is attempted but is known to be imperfect because of obvious effects related to study intervention or control product in some subjects (eg, dry mouth, bradycardia, fever, injection site reactions, and changes in laboratory data). Such problems or potential problems should be identified and, if there are plans to assess the magnitude of the problem or manage it, these should be described (eg, having endpoint measurements carried out by study staff shielded from information that might reveal study group assignment).

Explain whether blinding is considered unnecessary to reduce bias for some or all of the observations; eg, use of a random-zero sphygmomanometer eliminates possible observer bias in reading blood pressure and Holter tapes are often read by automated systems that are presumably immune to observer bias.

Describe the reasons and implications if blinding is considered desirable but not feasible. Sometimes blinding is attempted even though it is known to be imperfect because of obvious drug effects in at least some subjects (eg, dry mouth, bradycardia, fever, injection site reactions, or changes in laboratory data). Identify such problems or potential problems and discuss whether there will be any attempts to assess the magnitude of the problem or to manage it (eg, by having some endpoint measurements carried out by people shielded from information that might reveal treatment assignment).

5.7 Concomitant Therapy

Include content in this section if applicable, otherwise note as not applicable.

Example text provided as a guide, customize as needed:

[For this protocol, a prescription medication is defined as a medication that can be prescribed only by a properly authorized/licensed clinician. Medications to be reported in the case report form (CRF) are concomitant prescription medications, over-the-counter medications and supplements.]

Define the point in time at which ongoing therapy is considered to be concomitant, eg, at randomization or from start of study drug.

Provide the following information:

- Which medication(s)/treatment(s) will be permitted during the study (including rescue medication),
- Whether and how concomitant medications will be recorded, and
- Any other specific rules and procedures related to permitted or forbidden concomitant therapy, eg, whether specific concomitant medications would result in a subject's discontinuation from the study.

Note: It is important to avoid ambiguities regarding concomitant therapy, to minimize the possibility that a subject's data has to be excluded from the analysis.

If applicable, describe how allowed concomitant therapy might affect the outcome (eg, by drug-drug interaction or direct effects on the study endpoints) and how the independent effects of concomitant and study therapies could be ascertained.

If the list of concomitant medications is long, create a table and add it to a protocol appendix. Refer to appendix in the text.

5.8 Restrictions

Include content in this section if applicable, otherwise note as not applicable.
Describe any restrictions during any parts of the study pertaining to lifestyle and/or diet (eg, food and drink restrictions, timing of meals relative to dosing, intake of caffeine, alcohol, or tobacco, or limits on activity), and considerations for household contacts. Describe what action will be taken if prohibited medications, treatments or procedures are indicated for care (eg, early withdrawal).

5.8.1 Prior Therapy

Include content in this section if applicable, otherwise note as not applicable.

Describe prior therapy subjects may have undergone or were not allowed to have prior to enrollment in the study.

5.8.2 Fluid and Food Intake

- Specify any requirements or restrictions for fasting or for fluid and food intake during the study. Describe how those restrictions will be monitored, if applicable.

5.8.3 Subject Activity Restrictions

- Specify any requirements or restrictions for subject activities or confinement during the study. Describe how those restrictions will be monitored, if applicable.

5.9 Treatment Compliance

Define how adherence to the protocol (eg, administration of study intervention, use of device,) will be assessed, and verified (if applicable, eg, plasma assays, electronic monitoring devices, daily diaries). Include a discussion of what documents are mandatory to complete (eg, subject drug log) and what source documents/records will be used to calculate study intervention compliance.

Describe the measures that will be taken to ensure and document compliance with study dosing regimen, eg, drug accountability, diary cards, blood, urine, or other body fluid drug level measurements, or medication event monitoring.

5.10 Packaging and Labeling

- Provide details of the package type and quantity and labeling contents for the IP(s).
- Describe how any comparator(s) will be made to look like the IP(s) if anyone in the study is to be blinded.
- Describe how the label will be attached to the IP(s) and comparator(s) and whether it will be removed from the source for attachment to the CRF.

5.11 Storage and Accountability

5.11.1 Storage

Describe storage and stability requirements (eg, protection from light, temperature, humidity) for the study intervention and control product. For studies in which multi-dose vials are utilized, provide additional information regarding stability and expiration time after

initial use (eg, the seal is broken).Describe processes for secure storage of study treatment. Suggested text below:

[Drug supplies must be stored in a secure, limited-access location under the storage conditions specified on the label. Refer to the Pharmacy Manual for additional guidance on study drug storage.]

5.11.2 Accountability

State how the study intervention and control product will be provided to the Investigator. Describe plans about how and by whom the study intervention will be distributed, including participation of a drug repository or pharmacy, and plans for disposal of expired or return of unused product. Detailed information may be provided in a MOP or a separate SOP. Suggested text below, edit as needed:

[The Investigator or his/her designee will maintain accurate records of the receipt of all study drugs. In addition, accurate records will be kept regarding when and how much of each study drug is dispensed, used and unused by each study subject. Study center personnel should not dispose of any used or unused study drug until drug accountability is performed by the Sponsor or its designee. At the conclusion of the study, any unused study drug will be returned to the Sponsor or destroyed by the site pursuant to written authorization by the Sponsor and applicable federal and state regulations.]

5.11.3 Dispensing Study Drug

Describe the preparation of the study intervention and control product, including any preparation required by study staff and/or study subjects. Include thawing, diluting, mixing, and reconstitution/preparation instructions in this section, as appropriate, or within Appendix 2, MOP or SOP.

Provide the following information:

- The person(s) responsible for dispensing the study drug at the study site.
- Instructions for preparing the study drug, if needed.
- The procedures to be followed for dispensing the study drug.
- Instructions for safe handling and safe disposal, when applicable.

5.11.4 Investigational Product Retention at Study Site

Specify any requirements for retention of IP as per federal regulations. This section is applicable to bioequivalence and bioavailability studies. If there is no BA/BE component to the study (likely for most phase 2-3 studies), delete this section.

6 STUDY PROCEDURES

List and describe all study procedures and evaluations to be done as part of the study to support the determination of efficacy, as per the primary and secondary objectives outlined in this protocol. Discuss the sequence of events that should occur during the screening process and any decision points regarding subject eligibility. Include the time frame prior to enrollment within which screening procedures/ evaluations must be performed (eg, within 28 days prior to enrollment). If a separate screening protocol is developed, describe how the screening protocol will be used to identify subjects for this study.

In addition, discuss any special conditions that must be achieved during the enrollment and/or initial administration of study intervention. Include the procedures for administering the study intervention and follow-up procedures after administration (eg, assessment of vital signs), as well as any specifics about subsequent follow-up visits, and unscheduled visits. Also, note if a specifically qualified person (eg, physician, psychologist) should be performing any of the assessments, procedures/evaluations. Include any definitions used to characterize outcomes (eg, criteria for determining occurrence of acute myocardial infarction, characterization of a stroke as thrombotic or hemorrhagic, distinction between transient ischemic attack and stroke), should be explained fully.

Note that the protocol should provide a high-level discussion of all procedures and detailed information can be further provided in a MOP or SOP. Provide justification for any sensitive procedures (eg, provocative testing, deception). In addition, note where approaches to decrease variability, such as centralized laboratory assessments, are being employed. The specific timing of procedures/evaluations to be done at each study visit are captured in the SOE and the time points of these procedures do not need to be included here.

- Describe all study procedures to be performed for the study.
- Modify these suggested subheadings as applicable for the study.

6.1 Informed Consent

Describe how informed consent will be administered. Describe any proposed waivers or alterations to informed consent. Describe any special circumstances regarding obtaining consent. Describe plans for obtaining consent from speakers of language other than English. Describe procedures for obtaining surrogate consent for those unable to consent on their own behalf. This section should be consistent with Section 4.4, Strategies for Recruitment and Retention when describing consent plans and special considerations for children or other vulnerable subjects. Address re-consent processes for children who become adults or emancipated during a study.

Example text provided as a guide, customize as needed:

[Informed consent is a process that is initiated prior to the individual's agreeing to participate in the study and continues throughout the individual's study participation. Consent forms will be Institutional Review Board (IRB)-approved and the subject will be asked to read and review the document. The Investigator will explain the research study to the subject and answer any questions that may arise. A verbal explanation will be provided in terms suited to the subject's comprehension of the purposes, procedures, and potential risks of the study and of their rights as research participants. Subjects will have the opportunity to carefully review the written consent form and ask questions prior to signing. The subjects should have the opportunity to discuss the study with their family or surrogates or think about it prior to agreeing to participate. The subject will sign the informed consent document prior to any procedures being done specifically for the study. Subjects must be informed that participation is voluntary and that they may withdraw from the study at any time, without prejudice. A copy of the informed consent document will be given to the subjects for their records. The informed consent process will be conducted and documented in the source document (including the date), and the form signed, before the subject undergoes any study-specific procedures. The rights and welfare of the subjects will be protected by emphasizing to them that the quality of their medical care will not be adversely affected if they decline to participate in this study.]

6.2 Screening and Baseline Assessments

Describe the procedures or assessments to be done at Screening and Baseline as applicable to the study. Suggested headings are below:

6.2.1 Medical History

Provide the following information:

- The means for collecting and recording medical history information,
- The type of information to be collected and any timeframe for the history relative to study screening or startup, if applicable,
- A list of medications/treatments that will be permitted or must be discontinued within a certain time period before randomization or the start of study therapy,
- Whether and how prior/ongoing therapies will be recorded, and
- Any other specific rules and procedures related to permitted or forbidden prior/ongoing therapy.

Note: In most instances, prior therapies (taken before randomization or start of study therapy) should be differentiated from concomitant therapies (taken after start of study therapy) to facilitate understanding of any AEs that occur after the start of study therapy.

6.3 Efficacy Assessments

- Provide a list of the assessments for which efficacy endpoints will be calculated. Describe the assessments under subheadings as necessary.

6.4 Safety Assessments

Describe the safety assessments to be conducted during the study. Suggested headings are below.

6.4.1 Physical Examination

This could include height and weight, organ systems, motor or vision assessment, or other functional abilities. If appropriate, discuss what constitutes a targeted physical examination.

Provide the following information:

- The person(s) responsible for performing a physical examination of the subject,
- Whether the examination should be a complete or partial physical examination,
- The body systems to be included in the examination (eg, some studies do not warrant genitourinary examinations), and
- Whether the examination will include height, weight, body mass index, etc.

6.4.2 Vital Signs

This could include temperature, pulse, respirations, blood pressure. Carefully consider which vital signs (if any) should be measured to ensure that only essential data are collected. Include any specific instructions with respect to the collection and interpretation of vital signs.

Provide the following information:

- The person(s) responsible for performing a physical examination of the subject,
- Specifics on which assessments will be performed to monitor vital signs, ie, blood pressure, heart rate, respiratory rate, and/or temperature, and
- The method for consistent monitoring of vital signs, including:
  - Whether the subject will have to assume a certain body position each time (supine, sitting, standing, etc.),
  - Whether the blood pressure should be measured using the same arm and/or the same equipment each time, and/or

- Whether any mechanical devices will be used to monitor vital signs, and if so, whether and how they are calibrated for consistency across sites.

Note: if orthostatic assessment of vital signs is planned (measured after 5-10 minutes at rest in supine position, then 2-3 minutes after standing), the protocol should include a definition of orthostatic hypotension (SBP and DBP criteria) and, if applicable, abnormal HR increase.

6.4.3 Electrocardiography and Continuous Telemetry Monitoring

6.4.3.1 12-Lead Electrocardiograms

Specify if the electrocardiogram (ECG) is for screening purposes only. Include any specific instructions for the collection and interpretation of the ECG (eg, time points relative to dosing with study intervention or other evaluations). If ECGs will be analyzed at a central laboratory, instructions for the collection (eg, equipment), transmission and archiving of the ECG data should be summarized in this protocol, and further outlined in the MOP. If the ECG will be read locally, indicate how these will be handled and in what format (eg, digital or paper), as well as instructions with respect to local review.

6.4.3.2 Continuous Holter Monitoring

6.4.4 Clinical Laboratory Tests

6.4.4.1 Laboratory Parameters

Include specific test components and estimated volume and type of specimens needed for each test. Specify laboratory methods to provide for appropriate longitudinal and cross-sectional comparison (eg, use of consistent laboratory method throughout study, use of single, central laboratory for multi-site studies). If more than one laboratory will be used, specify which evaluations will be done by each laboratory. In addition, compliance with Clinical Laboratory Improvement Amendments (CLIA) of 1988 should be addressed. If such compliance is not required, a brief discussion should be included explaining why this is the case. In addition, discussion should include whether any laboratory tests (eg, diagnostics) that will be used are being developed concurrently or are commercially available. Describe:

- The type of clinical laboratory tests to be performed at any time during the study. This information can be presented concisely in a tabular format.
- Whether any laboratory parameters will be defined by the Sponsor as “clinically significant,” based on a predetermined range of laboratory values which will also be provided in this section.

- The conditions under which laboratory testing will occur, eg, timing in relation to meals and study dosing and frequency.

Example text provided as a guide, customize as needed:

[Subjects will be in a seated or supine position during blood collection. Clinical laboratory tests will include the following parameters listed in Table 1.]

Table 1 Clinical Laboratory Parameters

(Modify the following table as appropriate for the study.)

Measurement	Tests to be Performed
Hematology	Hematocrit (Hct) Hemoglobin (Hgb) Mean corpuscular hemoglobin (MCH) Mean corpuscular hemoglobin concentration (MCHC) Mean corpuscular volume (MCV) Platelet count Red blood cell (RBC) count White blood cell (WBC) count with differential
Urinalysis	Appearance Bilirubin Color Glucose Ketones Microscopic examination of sediment Nitrite Occult blood pH Protein Specific gravity Urobilinogen 24-hour urine for creatinine clearance Urine human chorionic gonadotropin (hCG) (only for females who are not diagnosed as postmenopausal)
Serum Chemistry	Albumin (ALB) Alkaline phosphatase (ALK-P) Alanine aminotransferase (ALT) Aspartate aminotransferase (AST) Blood urea nitrogen (BUN) Calcium (Ca) Carbon dioxide (CO_2) Chloride (Cl) Creatinine Creatine kinase and subtypes Gamma-glutamyl transferase (GGT) Globulin Glucose Lactate dehydrogenase (LDH) Phosphorus Potassium (K)

	Sodium (Na) Total bilirubin Direct bilirubin Total cholesterol Total protein Triglycerides Troponins Uric acid
Virus Testing	HBsAg, HCV, and HIV
Coagulation	Prothrombin time (PT) Activated partial thromboplastin time (aPTT)

Abbreviations: define abbreviations in table as applicable to study

6.4.4.2 Sample Collection, Storage, and Shipping

Special instructions for the preparation, handling, storage, and shipment of specimens should be briefly explained in this section with detailed discussion in the study's MOP.

Provide the following information:

- The person(s) responsible for collecting laboratory samples,
- The name and address of the laboratory that will process the samples, and
- Any special sample handling, labeling, or shipping procedures required.

6.4.5 Pregnancy Testing

- Include the defined timepoints for pregnancy testing in women of childbearing potential (WOCBP) in the SOE. Timepoints should be based upon a risk assessment of the potential for genotoxicity and teratogenicity/fetotoxicity of the intervention(s) in the study. Risk should be determined for each intervention with input from the company's nonclinical safety assessment group. Determination of risk for a marketed compound should also consider the risks outlined in the product label. Further guidance can be found in ICH Guideline M3(R2) and Clinical Trial Facilitation Group (CTFG). As a minimum, a pregnancy test should be performed at screening to confirm absence of pregnancy and at the end of relevant systemic exposure. Additional testing may be required between the screening visit and the first dose of study intervention on Day 1. Consider additional pregnancy testing if the interval is:
    - ≤ 4 days, a repeat highly sensitive serum pregnancy test usually is not required
    - > 4 days, a repeat serum pregnancy test should be obtained
- For studies that have requirements for multiple pregnancy tests, add additional criteria as needed (eg, if there is a requirement for a test to be performed within a proximal timeframe prior to first dose, specify as inclusion criteria; if at a specified visit, or at the end of study intervention, note in the SOE and provide any necessary details here).

- A serum pregnancy test may diagnose pregnancy ~6 to 10 days after fertilization; a urine pregnancy test, because it is less sensitive, will diagnose pregnancy a few days after a serum pregnancy test. As serum pregnancy tests have a lower detection limit and will detect pregnancy closer to the date of conception, serum testing is the preferred test if there is a requirement to know pregnancy status within a few days of the first dose of study intervention.
- Decide if local or central testing will be standard for the protocol. Highly sensitive serum testing is mandatory if required by local regulations or the IRB/IEC, or to resolve an indeterminate test or confirm a positive urine test.

6.5 Adverse Events

AE definitions are provided in Appendix 3.

Provide the following information:

- The person(s) responsible for assessing AEs and
- The method to be used to solicit information from the subject regarding AEs, eg, questions from study personnel, volunteered information, or subject questionnaire or diary form.

6.5.1 Time Period and Frequency for Collecting Adverse Event Information

Describe how AEs and SAEs will be identified and followed until resolved or considered stable. Specify procedures for recording and follow-up of AEs and SAEs that are consistent with the information contained within Section 8.2, Safety and Other Assessments including what assessment tools will be used to monitor AEs. Include duration of follow-up after appearance of events (eg, 1 week, 2 months).

An unsolicited AE would occur without any prompting or in response to a general question such as "Have you noticed anything different since you started the study; began the study intervention, etc." A solicited AE is one that is specifically solicited such as "Have you noticed any dry mouth since you started the study medication?"

- Describe which AEs will be collected as solicited events. Plan the reporting and data collection system to avoid double capture (captured both as an unsolicited and a solicited AE).
- Describe how unsolicited events will be captured.
- Include time period of collection (eg, Days 1-28) and note how long SAEs are collected – usually collected through entire study.
- Define the period during which a subject's medical complaint will be considered to be an AEs, eg, from the time study drug is first taken through the follow-up visit and

- Define the period during which a subject's medical complaint will be considered to be a treatment-emergent adverse event (if applicable).

Example text provided as a guide, customize as needed:

[The occurrence of an AE or SAE may come to the attention of study personnel during study visits and interviews of a study subject presenting for medical care, or upon review by a study monitor.

All AEs including local and systemic reactions not meeting the criteria for SAEs will be captured on the appropriate CRF. Information to be collected includes event description, time of onset, clinician's assessment of severity, relationship to study product (assessed only by those with the training and authority to make a diagnosis), and time of resolution/stabilization of the event. All AEs occurring while on study must be documented appropriately regardless of relationship. All AEs will be followed to adequate resolution.

Any medical condition that is present at the time that the subject is screened will be considered as baseline and not reported as an AE. However, if the study subject's condition deteriorates at any time during the study, it will be recorded as an AE.

Changes in the severity of an AE will be documented to allow an assessment of the duration of the event at each level of severity to be performed. AEs characterized as intermittent require documentation of onset and duration of each episode.

<Insert role or name> will record all reportable events with start dates occurring any time after informed consent is obtained until 7 (for non-serious AEs) or 30 days (for SAEs) after the last day of study participation. At each study visit, the Investigator will inquire about the occurrence of AE/SAEs since the last visit. Events will be followed for outcome information until resolution or stabilization.]

6.5.2 Method of Detecting Adverse Events and Serious Adverse Events

Suggested text is below, customize as needed:

[Care will be taken not to introduce bias when detecting AEs and/or SAEs. Open-ended and nonleading verbal questioning of the subject is the preferred method to inquire about AE occurrences.]

6.5.3 Follow-Up of Adverse Events and Serious Adverse Events

After the initial AE/SAE report, the Investigator is required to proactively follow each subject at subsequent visits/contacts. All SAEs (and adverse events of special interest [AESIs] as described in Section 6.5.10) will be followed until resolution, stabilization, the event is otherwise explained, or the subject is lost to follow-up (as defined in Section 6.9.3). Further information on follow-up procedures is provided in Appendix 3.

6.5.4 Reporting Serious Adverse Events

This section addresses responsibilities of Investigators for reporting of AEs. However, it is important to recognize that Sponsors have additional responsibilities under regulations that are not described in this template and should be incorporated into relevant SOPs.

Describe the AE reporting procedures, including timeframes. Further details should be included in a MOP or SOP including a description and a flow chart of when events are reported to various oversight (eg, Data and Safety Monitoring Board (DSMB), safety monitoring committee, independent safety monitor) and regulatory groups, and what study staff are responsible for completing and signing off on the AE reports, and who will receive notification of AEs. According to 21 CFR 312.64(b), "…The Investigator must record non-serious AEs and report them to the Sponsor according to the timetable for reporting specified in the protocol".

In addition, list any disease-related events (DREs) common in the study population (eg, expected), which will not be reported per the standard process for reporting, as applicable. Describe how these events will be recorded and monitored.

- Define how soon and to whom the SAE should be reported, eg, to the ethics committee, regulatory authorities, and Sponsor.
- Define how to report them, eg, fax, phone, email, regular mail.
- Provide the name and contact information for immediate reporting.
- Describe whether a form will be used to report SAEs and what information should be included in the report.

Describe the SAE reporting procedures, including timeframes. Further details should be included in a MOP or SOP including a description and a flow chart of when events are reported to various oversight and regulatory groups, and what study staff are responsible for completing and signing off on the SAE reports, and who will receive notification of SAEs.

Example 1, applicable for a drug or biologic protocol:

[The study clinician will immediately report to the Sponsor any SAE, whether or not considered study intervention related, including those listed in the protocol or Investigator brochure and must include an assessment of whether there is a reasonable possibility that the study intervention caused the event. Study endpoints that are SAEs (eg, all-cause mortality) must be reported in accordance with the protocol unless there is evidence suggesting a causal relationship between the study intervention and the event (eg, death from anaphylaxis). In that case, the Investigator must immediately report the event to the Sponsor.

All SAEs will be followed until satisfactory resolution or until the site Investigator deems the event to be chronic or the subject is stable. Other supporting documentation of the event may be requested by the Data Coordinating Center (DCC)/study Sponsor and should be provided as soon as possible.

The study Sponsor will be responsible for notifying the Food and Drug Administration (FDA) of any unexpected fatal or life-threatening suspected adverse reaction as soon as possible, but in no case later than 7 calendar days after the Sponsor's initial receipt of the information. In addition, the Sponsor must notify FDA and all participating Investigators in an Investigational New Drug (IND) safety report of potential serious risks, from clinical trials or any other source, as soon as possible, but in no case later than 15 calendar days after the Sponsor determines that the information qualifies for reporting.]

6.5.5 Institutional Review Board/Independent Ethics Committee Notification

Suggested text for IRB/IEC notification of reporting SAEs, modify as needed for the study.

[The Investigator must report all SAEs (including follow-up information) to the IRB/IEC in accordance with IRB/IEC requirements. Copies of each report and documentation of IRB/IEC notification and receipt will be kept in the site's Regulatory Binder.]

6.5.6 Clinical Significance

- Identify the person(s) responsible for determining whether an AEs is clinically significant for the subject or the study overall.
- Describe the criteria for determining clinical significance and the method for documenting clinical significance.

Suggested text is below, modify as needed for the study.

[The Investigator and/or qualified designee are responsible for monitoring AEs/SAEs and determining clinical significance using Common Terminology Criteria for Adverse Events (CTCAE) grading. Each AE/SAE will be evaluated as described in Appendix 3.]

6.5.7 Clinical Laboratory Adverse Events

- Specify whether abnormal laboratory test results will be considered to be AEs, and if so, define the cut-off values or other conditions under which they will be considered AEs, eg, they are associated with clinical signs and symptoms, they require treatment or diagnostic procedures, they result in discontinuation from the study, or they are associated with death or an SAE.

6.5.8 Serious Adverse Events

The definition of an SAE and instructions for SAE follow-up are provided in Appendix 3.

6.5.9 Treatment-emergent Adverse Events

Include content in this section if applicable, otherwise note as not applicable. Example text provided as a guide, customize as needed:

[Information regarding the occurrence of treatment-emergent adverse events (TEAEs) will be captured immediately following study drug administration and throughout the study, verified at each study visit. A description of the AE, duration (start and stop dates and times), severity, outcome, treatment, and relation to study medication will be collected.]

6.5.10 Adverse Events of Special Interest

Include content in this section if applicable, otherwise note as not applicable.

Describe any other events that merit reporting to the Sponsor, study leadership, IRB, and regulatory agencies. For example, in oncology trials, secondary malignancies are often captured.

Include any other reportable events not already included in the previous sections, such as cardiovascular and death events, medical device incidents (including malfunctions), laboratory test abnormalities, and study intervention overdose.

6.5.11 Suspected Unexpected Serious Adverse Reactions

Include content in this section if applicable, otherwise note as not applicable. Example text provided as a guide, customize as needed:

[The study Sponsor is required to inform FDA of any of any suspected unexpected serious adverse reaction (SUSAR). Unexpected AEs are those that are not identified in nature, severity, or frequency in the current Investigator's Brochure, protocol, or informed consent form. All SUSARs will be the subject of expedited reporting. The reporting timeline for a SUSAR that has occurred inside or outside the US is as follows:

- Where it is neither fatal nor life-threatening, within 15 days after becoming aware of the information.
- Where it is fatal or life-threatening, immediately where possible and, in any event, within 7 days after becoming aware of the information.
- Further, within 8 days of having informed FDA of a fatal or life-threatening SAE, the Sponsor must submit as complete a report as possible, to include an assessment of the importance and implication(s) of any findings. Follow-up reports of fatal or life-threatening reactions must include an assessment of the importance and implication of the findings, including relevant previous experience with the same or similar drugs.
- Any relevant follow-up information is communicated within an additional 15 calendar days after becoming aware of the information.]

6.6 Pregnancy Reporting

Include content in this section if applicable, otherwise note as not applicable. Pregnancy is not an AEs, but some studies will require unique considerations if pregnancy was to occur during the study.

State the study's pregnancy-related policy and procedure. Include appropriate mechanisms for reporting to the DCC or NIH, the IND or IDE Sponsor, study leadership, IRB, and regulatory agencies. Provide appropriate modifications to study procedures (eg, discontinuation of study intervention, while continuing safety follow-up, requesting permission to follow pregnant women to pregnancy outcome).

Add the rule for reporting pregnancies during the study, or state not applicable. The following text is suggested, modify as needed for the study.

[Should a female subject or female partner of a male subject become pregnant at any time during the study, the Investigator must notify the Sponsor within 24 hours after learning about the pregnancy. The Sponsor will request follow-up information regarding the course of the pregnancy and its outcome. Voluntary consent must be obtained from a female subject or female partner of a male subject (and/or legally authorized representative if the subject is mentally incompetent or physically incapacitated) before any pregnancy-related follow-up is obtained.

Pregnancy, in and of itself, is not considered an AE, unless there is suspicion that the study product may have interfered with the effectiveness of a contraceptive medication or method. However, any pregnancy complication or elective termination of a pregnancy for medical reasons will be recorded as an AE or SAE and will be followed accordingly. A spontaneous abortion is an SAE.]

6.7 Definition of Overdose and Reporting of Overdose

- Define the parameters for drug overdose if applicable to the study.
- Describe the process of reporting study treatment overdose to the Sponsor.

Suggested text below, modify as applicable.

[For the purposes of this study, an overdose of {product name} will be defined as any dose of <dose level>. No specific information is available on the treatment of overdose of {product name}/ OR <describe treatment for overdose if known>. In the event of an overdose, the subject should be observed closely for signs of toxicity. Appropriate supportive treatment should be provided if clinically indicated.]

If an AE is associated with, or results from the overdose of {product name}, the AE is reported as an SAE, even if no other seriousness criteria are met.

All reports of overdose with and without an AE must be reported within 24 hours to the Sponsor.

6.8 Concomitant Medication Assessments

Provide the following information:

- The person(s) responsible for assessing and recording the subject's concomitant medication use.
- The method for recording concomitant medication use.

Example text provided as a guide, customize as needed:

[Concomitant medication information will be collected at every study visit by the study Investigator or qualified study personnel. During the course of study treatment, if an enrolled subject is prescribed an exclusionary medication, the subject …]

6.9 Removal of Subjects from the Trial or Study Drug

Example text provided as a guide, customize as needed:

[A subject may choose to discontinue study treatment and/or withdraw from the study at any time. Withdrawal of consent will not affect the subject's future care by the Investigator or study site. In the absence of a medical contraindication, every effort should be made by the Investigator to encourage subjects who prematurely discontinue treatment to remain in the study as appropriate and, at a minimum, follow the subject for safety data collection.]

6.9.1 Discontinuation of Study Drug

Example text provided as a guide, customize as needed:

[Discontinuation from <study intervention> does not mean discontinuation from the study, and remaining study procedures should be completed as indicated by the study protocol. If a clinically significant finding is identified after enrollment (including, but not limited to changes from baseline), the Investigator or qualified designee will determine if any change in subject management is needed. Any new clinically relevant finding will be reported as an AE.

The data to be collected at the time of study intervention discontinuation will include the following:

Describe the procedures and data to be collected, as well as any follow-up evaluations

An Investigator may discontinue or withdraw a subject from the study for the following reasons:

- Completion of study intervention.
- Disease progression which requires discontinuation of the study intervention.
- If any clinical AE, laboratory abnormality, or other medical condition or situation occurs such that continued participation in the study would not be in the best interest of the subject.
- Investigator discretion.
- Positive pregnancy test.
- Subject unable to receive <study intervention> for [x] days/weeks.]

6.9.2 Withdrawal From the Study

Example text provided as a guide, customize as needed:

When and how to withdraw subjects from the trial/IP(s) dosing regimen,

- The type and timing of the data to be collected for withdrawn subjects,
- Whether and how subjects are to be replaced, and
- What procedures the site should follow to notify the Sponsor when a subject is discontinued from the study.

[The Investigator may withdraw a subject from the study for any of the following reasons:

- A protocol violation occurs.
- A serious or intolerable AE occurs.
- A clinically significant change in a laboratory parameter occurs.
- The Sponsor or Investigator terminates the study.
- The subject requests to be discontinued from the study.

Subjects who withdraw or are withdrawn from the study prematurely should be contacted at the earliest opportunity to: 1) complete an early termination evaluation (review of AEs, concomitant therapies) and 2) undergo lab testing for safety monitoring purposes (as outlined in the other study visits).

The reason for subject discontinuation or withdrawal from the study will be recorded on the CRF. Subjects who sign the informed consent form and are randomized but do not receive the study intervention may be replaced. Subjects who sign the informed consent form, and are randomized and receive the study intervention, and subsequently withdraw, or are withdrawn or discontinued from the study, be replaced.]

6.9.3 Lost to Follow-Up

Example text provided as a guide, customize as needed:

[A subject will be considered lost to follow-up if he or she fails to return for <specify number of visits> scheduled visits and is unable to be contacted by the site staff.

The following actions must be taken if a subject fails to return to the clinic for a required study visit:

The site will attempt to contact the subject and reschedule the missed visit <specify time frame> and counsel the subject on the importance of maintaining the assigned visit schedule and ascertain if the subject wishes to and/or should continue in the study.

Before a subject is deemed lost to follow-up, the Investigator or designee will make every effort to regain contact with the subject (where possible, 3 telephone calls and, if necessary, a certified letter to the subject's last known mailing address or local equivalent methods). These contact attempts should be documented in the subject's medical record or study file.

Should the subject continue to be unreachable, he or she will be considered to have withdrawn from the study with a primary reason of lost to follow-up.]

6.10 Study Stopping Rules

List possible reasons for termination or temporary suspension of the study (eg, study closure based on PI decision, Sponsor/funder decision, regulatory or other oversight bodies; review of serious, unexpected, and related AEs; noncompliance; futility). For any study that is prematurely terminated or temporarily suspended, the PI will promptly inform study subjects, the IRB, and Sponsor and provide the reason(s) for the termination or temporary suspension.

When a study is prematurely terminated, refer to Section 6, Study Intervention Discontinuation and Participant Discontinuation/Withdrawal, for handling of enrolled study subjects.

Example text provided as a guide, customize as needed:

[This study may be temporarily suspended or prematurely terminated if there is sufficient reasonable cause. Written notification, documenting the reason for study suspension or termination, will be provided by the suspending or terminating party to <study subjects, Investigator, funding agency, the IND or Investigational Device Exemption (IDE) Sponsor and regulatory authorities>. If the study is prematurely terminated or suspended, the Principal Investigator (PI) will promptly inform study subjects, the IRB, and Sponsor and will provide the reason(s) for the termination or suspension. Study subjects will be contacted, as applicable, and be informed of changes to the study visit schedule.

Circumstances that may warrant termination or suspension include, but are not limited to:

- Determination of unexpected, significant, or unacceptable risk to subjects.
- Demonstration of efficacy that would warrant stopping.

- Insufficient compliance to protocol requirements.
- Data that are not sufficiently complete and/or evaluable.
- Determination that the primary endpoint has been met.
- Determination of futility.

The study may resume once concerns about safety, protocol compliance, and data quality are addressed, and satisfy the Sponsor, IRB and/or FDA/other Health Authority.]

6.11 Other Study Procedures

- This section could contain exploratory endpoints, or quality of life questionnaires etc.

6.12 Appropriateness of Measurements

- Explain the appropriateness of measures to be used. If any of the safety assessments are not standard, ie, widely used and generally recognized as reliable, accurate, and relevant (able to discriminate between effective and ineffective agents), describe the measurement's reliability, accuracy, and relevance. It may be helpful to describe alternatives that were considered but rejected.
- Provide justification if a surrogate endpoint (a laboratory measurement or physical measurement or sign that is not a direct measure of clinical benefit) will be used as a study endpoint, eg, by reference to clinical data, publications, guidelines, or previous actions by regulatory authorities.

7 STUDY ACTIVITIES

DELETE THIS SECTION UNLESS IT IS SPECIFICALLY REQUESTED BY THE SPONSOR, AS IT IS ALWAYS REDUNDANT WITH THE SOE AND OFTEN REDUNDANT WITH SECTION 6.

Once this section is deleted, the numbering of the section headers will automatically update to show "QUALITY CONTROL AND ASSURANCE" as Section 7, etc.

If the Sponsor wants to include this section in the protocol, take the following steps to restore this section (the formatting, including section headings, will also be restored):

- Select the entirety of Section 7.
- Open the Font menu and uncheck the "Hidden" box.
- Change font to black ("Automatic").
- Un-italicize the text.

In this section, if it is included:

- Indicate that a study events table is presented in an appendix to the protocol.
- List the study procedures to be performed and the timing of those procedures for each study activity.
- Make sure that the timing of events in this section matches the SOE exactly.
- Provide a concise outline of the procedure(s) that will be performed at each visit, by time.
- Use consistent terminology when referring to procedures, visits, and times, eg, "Visit 1," "Day 1," or "Week 1."
- Describe the acceptable or unacceptable limits for visit windows.
- List specific timing for procedures in relation to meals or study treatment administration, if applicable, eg, 30 minutes before study treatment is given or within 10 minutes after study treatment is given.
- Modify these suggested subheadings as applicable for the study.

7.1 Screening Visit (Days –XX to -1)

- Describe the acceptable length of time for the screening period in addition to the instructions listed above.

-
-

7.2 Clinic Admission (Day -1)

7.3 Treatment Period (Day 1 to Day XX)

7.3.1 Visit 1 (Day 1) Procedures

-
-

7.3.2 Visit 2 (Day XX) Procedures

-
-

7.4 Day XX or Early Termination Procedures

- List the procedures to be performed for early termination assessments, if there are differences in the procedures to be performed when the subject is not completing the study. Otherwise, list the procedures in the final visit section if the same procedures apply.
- Describe actions that the study site personnel should take to track subjects after the study.
- Indicate what Sponsor notification procedures are to be followed in the event that a subject withdraws or is withdrawn from the study.

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7.5 Follow-up Visit (Day XX ± X)

- Describe the follow-up procedures and time period for subjects withdrawn from the trial and/or IP(s) treatment.
- Define any safety testing and follow-up of AEs that result in discontinuing the subject from the study or from the study drug, eg, until resolution, until the event has stabilized, or until the event has been determined to be caused by an agent other than the study drug.
- Note: Include a description of how the Sponsor plans to continue to monitor any discontinued subjects who become pregnant during the study and the newborn, if applicable.
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8 QUALITY CONTROL AND ASSURANCE

- Describe the quality assurance and quality control systems that will be implemented to assure the quality of the data.
- Describe any steps that will be taken at the investigation site or centrally to ensure the use of standard terminology and the collection of accurate, consistent, complete, and reliable data, such as training sessions, monitoring of Investigators by Sponsor personnel, instruction manuals, data verification, cross-checking, use of a central laboratory for certain tests, centralized ECG reading, or data audits.
- Note whether Investigator meetings or other steps were taken to prepare Investigators and standardize performance.
- Note whether the Sponsor will use an independent internal or external auditing procedure, eg, study monitoring.

Example text provided as a guide, customize as needed:

[Each site will perform internal quality management of study conduct, data and biological specimen collection, documentation and completion. An individualized quality management plan will be developed to describe a site's quality management.

Quality control (QC) procedures will be implemented beginning with the data entry system and data QC checks that will be run on the database will be generated. Any missing data or data anomalies will be communicated to the site(s) for clarification/resolution.

Following written standard operating procedures (SOPs), the will verify that the clinical trial is conducted, data are generated, and biological specimens are collected, documented (recorded), and reported in compliance with the protocol, International Council for Harmonization (ICH) Good Clinical Practice (GCP), and applicable regulatory requirements (eg, Good Laboratory Practice [GLP], Good Manufacturing Practice [GMP]).

The site will provide direct access to all trial-related sites, source data/documents, and reports for the purpose of monitoring and auditing by the Sponsor, and inspection by local and regulatory authorities.]

9 PLANNED STATISTICAL METHODS

9.1 General Considerations

Provide the following information:

- Who will have responsibility for statistical analyses,
- The statistical methods to be employed, including timing of any planned interim analyses,
- The level of significance to be used,
- The method/software to be used for performing the analyses,
- Any definitions needed to explain the analyses, eg, baseline or change from baseline,
- Procedures for accounting for missing, unused, and spurious data,
- Procedures for reporting any deviation(s) from the original statistical plan, and
- Criteria for termination of the trial.

9.2 Determination of Sample Size

- Describe the number of subjects planned for enrollment and the number projected for each trial site.
- State the reason for choice of sample size, including reflections on (or calculations of) the power of the trial and clinical justification.
- Provide the planned sample size and the basis for it, such as statistical considerations or practical limitations.
- For a positive control study intended to show that a new therapy is at least as effective as the standard therapy, the sample size determination should specify the difference between treatments that would be considered unacceptably large and, therefore, the difference the study is designed to be able to exclude.

9.3 Analysis Populations

- Define the populations to be analyzed in the study.
- Define the selection of subjects to be included in the analyses (eg, all randomized subjects, all dosed subjects, all eligible subjects, and/or evaluable subjects).
- Describe any reasons why subjects for whom data are available might be excluded from the analysis.
- Identify any subgroups whose results are to be examined separately.
- Describe the way in which subject replacement will be addressed for the analysis (if applicable).

9.4 Demographics and Baseline Characteristics

- Describe the demographic and baseline characteristic data to be collected and the method of analysis for this data.

9.5 Efficacy Analysis

- Specify the study method or measurement that will be used to collect the data needed to determine whether the efficacy endpoints have been achieved.
- Define categorical responses (global scales, severity scores, responses of a certain size) that are to be used in analyzing responses (if any).
- Describe the statistical method that will be used to calculate and summarize each endpoint variable.
- Describe whether individual endpoints will be summarized and provided in tables, and if so, what will be included in those tables.
- Specify the particular measurements (eg, average of several measurements over the entire study, values at particular times, values only from study completers, or last on-therapy value) planned as the basis for comparison of test drug/IP and control, if critical measurements are to be made more than once.
- Similarly, identify the planned approach if more than one analytical approach is plausible, eg, changes from baseline response, slope analysis, life table analysis.
- Specify whether the primary analysis is to include adjustment for covariates.

9.6 Safety Analysis

- Include the coding system to be used for AEs (eg, MedDRA, COSTART).
- Specify the study method or measurement that will be used to collect the data needed to determine whether the safety endpoints have been achieved.
- Describe the statistical method that will be used to calculate and summarize each endpoint variable.

9.7 Other Assessments or Analyses

- Describe the planned statistical analyses for all other data collected during the study, eg, demographics and pretreatment characteristics or medications.
- Add or delete sections describing other assessments conducted, as necessary.

9.8 Interim Analysis

Describe the frequency and nature of any planned interim analyses, any specified circumstances in which the study would be terminated, and any statistical adjustments to be employed because of interim analyses.

10 ADMINISTRATIVE CONSIDERATIONS

10.1 Investigators and Study Administrative Structure

Provide the name and contact information of the PI and the Medical Monitor.

In addition, briefly describe any study leadership committees (eg, Steering Committee, Executive Committee, Subcommittee) and their roles. Note that it is not necessary to list specific members. Also, describe country-specific administrative requirements or functions that materially affect the conduct of the study. The MOP should include a list of study team roles and responsibilities of those involved in the conduct, management, or oversight of the trial.

- List the name, title, address, and phone number of the principal or coordinating Investigator who is responsible for overseeing the conduct of the trial. If multiple Investigators participated, this information may be placed in an appendix to this protocol.
- List the name, title, address, and telephone number of the qualified physician (or dentist, if applicable) who is responsible for all trial site-related medical decisions (if other than Investigator).
- Note that the subject's primary care physician is to be notified of study participation (if needed).
- List the names and addresses of the clinical laboratories and other medical or technical departments or institutions involved in the trial.
- List the names, titles, addresses, and other relevant contact information not described above (eg, for a steering committee, data monitoring committee, contract research organization, or clinical trial supply management source.)

10.2 Institutional Review Board or Independent Ethics Committee Approval

- Indicate that the study and any amendments are to be reviewed by an appropriate IRB (United States) and/or IEC (Europe) as applicable.

- Delete either term (IRB or IEC) if inappropriate for the study or include both if applicable.
- Indicate that a letter from the IRB/IEC indicating approval of the Investigator was received by the study Sponsor.
- Indicate that all reviews and approval by IRB/IEC will be in accordance with GCP.
- Note the requirement to notify the IRB/IEC of any SAEs.

Example text provided as a guide, customize as needed:

[Federal regulations and the ICH guidelines require that approval be obtained for each clinical site from an IRB/IEC before participation of human subjects in research studies. Before study onset, the protocol, informed consent form, and any other written information regarding this study to be provided to the subject or the subject's legal guardian must be approved by the IRB/IEC, and written confirmation of such approval must be received by the site. Documentation of all approvals and of the IRB/IEC compliance with ICH Guidelines for GCP E6 (R2) will be maintained by the site and will be available for review by the Sponsor or its designees.

All approvals should be signed by the IRB/IEC chair or designee and must identify the IRB/IEC by name and address, the clinical protocol by title or protocol number or both, and the date approval or favorable opinion was granted.]

10.3 Ethical Conduct of the Study

- Confirm that the study will be conducted in accordance with the ethical principles that have their origins in the Declaration of Helsinki.

Example text provided as a guide, customize as needed:

[The Investigators will ensure that this study is conducted in full conformity with the Declaration of Helsinki, the ICH GCP regulations and guidelines, principles of The Belmont Report: Ethical Principles and Guidelines for the Protection of Human Subjects of Research, National Commission for the Protection of Human Subjects of Biomedical and Behavioral Research (April 18, 1979), and codified in the United States (US) Code of Federal Regulations (CFR), title 45 CFR 46, 21 CFR 50 and 56, and ICH E6 (R2); 62 Federal Regulations 25692 (1997), as applicable.]

10.4 Subject Information and Consent

- Describe how and when informed consent will be obtained in relation to subject enrollment (eg, at allocation, pre-screening).
- Describe how informed consent will be documented.
- Indicate that the informed consent and any other written study materials provided to the subject were approved by an IRB/IEC before subject enrollment.

- Describe any other written study materials to be provided to the subjects.

Example text provided as a guide, customize as needed:

[Each subject will be provided a written informed consent form (ICF) and assent form (as applicable), describing this study and providing sufficient information to make an informed decision about participation in this study.

A copy of the proposed ICF and assent form must be submitted to the Sponsor and/or designee for review prior to submission to the reviewing IRB/IEC. The ICF and assent form must be approved by the IRB/IEC and must contain all elements required by federal, state, local, and institutional regulations or requirements. Institutions in the US must be in compliance with Health Insurance Portability and Accountability Act of 1996 (HIPAA) with regard to obtaining authorizations for use and disclosure of Protected Health Information.

The study, including its goals, methods, expected benefits, and potential hazards will be completely explained to each prospective study subject. It will also be explained to subjects that they are free to refuse entry or to withdraw from the study at any time without prejudice to future treatment. Voluntary informed consent or assent (for subjects < 18 years old) must be obtained from each eligible subject (and/or legally authorized representative if the subject is mentally incompetent or physically incapacitated) before any protocol-defined procedures are performed.

The subject's willingness to participate in this study will be documented on the IRB/IEC approved ICF and assent form (as applicable) which must be signed and dated by the subject or legally acceptable surrogate, and the physician Investigator (Principal or sub-Investigator). The Investigator will keep the original ICF and assent form (as applicable) and a copy will be given to the subject. The process for obtaining informed consent should also be noted in the subject's source documentation.]

10.5 Subject Confidentiality

This section will describe protections for maintaining confidentiality of subject data, including, but not limited to forms, records and samples and subject privacy.

Include procedures for maintaining subject confidentiality, privacy protections, any special data security requirements, and record retention per the Sponsor's requirements. Describe who would have access to records, including the Investigator and other study staff, the clinical monitor, funding institutions, representatives of the NIH Institute or Center (IC), IND/IDE Sponsor, representatives from the IRB, regulatory agencies, and representatives of the pharmaceutical company supplying product to be tested. In addition, consider inclusion of the following information:

- Describe whether identifiers will be attached to data/samples, or whether data will be coded or unlinked.

- If unlinked or coded, and additional information (eg, age, ethnicity, sex, diagnosis) is available, discuss whether this might make specific individuals or families identifiable.
- If research data/samples will be coded, describe how access to the "key" for the code will be limited. Include description of security measures (password-protected database, locked drawer, other). List names or positions of persons with access to the key.
- Include a discussion of the circumstances in which data or samples will be shared with other researchers.
- Include a discussion of plans to publish subject's family pedigrees, with a description of measures to minimize the chance of identifying specific families.
- Describe any situations in which personally identifiable information will be released to third parties.
- State who has access to records, data, and samples. Consider if monitors or auditors outside of study Investigators will need access.
- Discuss any additional features to protect confidentiality (eg, use of a certificate of confidentiality).
- Approaches to ensure privacy of study subjects.
- Describe the way in which subject confidentiality will be ensured.
- Provide information regarding the disclosure of protected health information as required under the Health Information Portability and Accountability Act (HIPAA) (if applicable).

Example text provided as a guide, customization will be required to address all aspects that should be included in this section:

[Subject confidentiality and privacy is strictly held in trust by the participating Investigators, their staff, and the Sponsor(s) and their interventions. This confidentiality is extended to cover testing of biological samples and genetic tests in addition to the clinical information relating to subjects. Therefore, the study protocol, documentation, data, and all other information generated will be held in strict confidence. No information concerning the study, or the data will be released to any unauthorized third party without prior written approval of the Sponsor.

All research activities will be conducted in as private a setting as possible.

The study monitor, other authorized representatives of the Sponsor, representatives of the IRB/IEC, regulatory agencies or pharmaceutical company supplying study product may inspect all documents and records required to be maintained by the Investigator, including but not limited to, medical records (office, clinic, or hospital) and pharmacy records for the subjects in this study. The clinical study site will permit access to such records.

The study subjects' contact information will be securely stored at each clinical site for internal use during the study. At the end of the study, all records will continue to be kept in a secure location for as long a period as dictated by the reviewing IRB/IEC, institutional policies, or Sponsor requirements.

Study subject research data, which is for purposes of statistical analysis and scientific reporting, will be transmitted to and stored at the <specify name of DCC>. This will not

include the subjects' contact or identifying information. Rather, individual subjects and their research data will be identified by a unique study identification number. The study data entry and study management systems used by clinical sites and by <specify name of DCC> research staff will be secured and password-protected. At the end of the study, all study databases will be de-identified and archived at the <specify name of DCC>.]

10.6 Study Monitoring

Site monitoring is conducted to ensure that the rights and well-being of trial subjects are protected, that the reported trial data are accurate, complete, and verifiable, and that the conduct of the trial is in compliance with the currently approved protocol/amendment(s), with ICH GCP, and with applicable regulatory requirement(s).

This section should give a general description of how monitoring of the conduct and progress of the clinical investigation will be conducted (ie, who will conduct the monitoring, the type, frequency, and extent of monitoring, who will be provided reports of monitoring, if independent audits of the monitoring will be conducted). This section may refer to a separate detailed clinical monitoring plan.

Example text when a separate CMP is being used is provided as a guide, customize as needed:

[Clinical site monitoring is conducted to ensure that the rights and well-being of trial subjects are protected, that the reported trial data are accurate, complete, and verifiable, and that the conduct of the trial is in compliance with the currently approved protocol/amendment(s), with ICH GCP, and with applicable regulatory requirement(s).

Monitoring for this study will be performed by .

Insert brief description of type of monitoring (eg, on-site, centralized), frequency (eg, early, for initial assessment and training versus throughout the study), and extent (eg, comprehensive (100% data verification) versus targeted or random review of certain data (less than 100% data verification or targeted data verification of endpoint, safety, and other key data variables)>

will be provided copies of monitoring reports within <x> days of a monitoring visit.

Details of clinical site monitoring are documented in a Clinical Monitoring Plan (CMP). The CMP describes in detail who will conduct the monitoring, at what frequency monitoring will be done, at what level of detail monitoring will be performed, and the distribution of monitoring reports.

Independent audits be conducted by to ensure monitoring practices are performed consistently across all participating sites and that monitors are following the CMP.]

- Identify the person(s) responsible for monitoring the study site and study activities.

- Describe the frequency and extent of monitoring to be performed during the study.
- Describe the planned monitoring of the results of the study.

10.7 Data Collection and Management Responsibilities

Provide details regarding the type(s) of data capture that will be used for the study and any relevant data standards or common data elements that are being utilized as a part of the trial. Specify whether it will be paper or electronic, distributed or central, batched or ongoing processing, and any related requirements. Indicate expectations for time for submission of CRFs. Further details should be provided in the MOP or the data management plan, including detailed descriptions of source documentation, CRFs, instructions for completing forms, data handling procedures, and procedures for data monitoring.

Source data are all information, original records of clinical findings, observations, or other activities in a clinical trial necessary for the reconstruction and evaluation of the trial. Electronic source data are data initially recorded in electronic form. Examples of source data include, but are not limited to, hospital records, clinical and office charts, laboratory notes, memoranda, subjects' memory aids or evaluation checklists, pharmacy dispensing records, audio recordings of counseling sessions, recorded data from automated instruments, copies or transcriptions certified after verification as being accurate and complete, microfiches, photographic negatives, microfilm or magnetic media, x-rays, and subject files and records kept at the pharmacy, at the laboratories, and medico-technical departments involved in the clinical trial. It is acceptable to use CRFs as source documents. If this is the case, it should be stated in this section what data will be collected on CRFs and what data will be collected from other sources.

It is not acceptable for the CRF to be the only record of a subject's inclusion in the study. Study participation should be captured in a subject's medical record. This is to ensure that anyone who would access the subject medical record has adequate knowledge that the subject is participating in a clinical trial.

Describe responsibilities for data handling and record keeping as they specifically relate to the IND/IDE Sponsor (if applicable), the award site, clinical site(s), laboratory(ies), and DCC. Information should include the role in data collection, review of data, trial materials, and reports, as well as retention of source documents, files, and records. Describe coding dictionaries to be used and reconciliation processes (if applicable).

If data are to be generated in one location and transferred to another group, describe the responsibilities of each party.

Provide a list of planned data standards, formats, terminologies and their versions, used for the collection, tabulation, analysis of study data. Refer to the FDA Guidance for Industry, Providing Regulatory Submissions in Electronic Format — Standardized Study Data, Study Data Technical Conformance Guide and FDA Guidance for Industry, Providing Regulatory

Submissions in Electronic Format - Human Pharmaceutical Product Applications and Related Submissions Using the eCTD Specifications.

Example text provided as a guide, customize as needed:

[Data collection is the responsibility of the clinical trial staff at the site under the supervision of the site Investigator. The Investigator is responsible for ensuring the accuracy, completeness, legibility, and timeliness of the data reported.

All source documents should be completed in a neat, legible manner to ensure accurate interpretation of data.

Hardcopies of the study visit worksheets will be provided for use as source document worksheets for recording data for each subject enrolled in the study. Data recorded in the electronic case report form (eCRF) derived from source documents should be consistent with the data recorded on the source documents.

Clinical data (including AEs, concomitant medications, and expected adverse reactions data) and clinical laboratory data will be entered into <specify name of data capture system>, a 21 CFR Part 11-compliant data capture system provided by the <specify DCC>. The data system includes password protection and internal quality checks, such as automatic range checks, to identify data that appear inconsistent, incomplete, or inaccurate. Clinical data will be entered directly from the source documents.]

10.8 Case Report Forms and Study Records

- Describe the type of source documents to be used for the study.
- Indicate how data will be recorded on the CRFs.
- Identify the CRF creator and supplier.
- Describe the procedures for correcting CRF entries.
- Provide instructions for transmittal of CRFs to the Sponsor (including method and timing).

Example text provided as a guide, customize as needed:

[All CRFs will be completed according to CRF completion guidelines. The Investigator must electronically sign and date a declaration on the CRF attesting to his/her responsibility for the quality of all data recorded and that the data represent a complete and accurate record of each subject's participation in the study. An electronic copy of the completed CRF will then be sent to the Investigator to be retained for their records.]

10.9 Data Monitoring Committee

If there is to be a data monitoring committee, either within or outside the Sponsor's control, its composition and operating procedures should be described and procedures to be used to maintain study blinding should be given.

Appropriate safety oversight should be used for each trial. This could include a Safety Monitoring Committee (SMC), DSMB, Safety Assessment Committee, and/or an Independent Safety Monitor (ISM). Independent oversight is an important component to ensure human subjects' protection and data integrity and should be considered for each study. In this section, the type of safety oversight should be clearly identified along with any known responsibilities for the oversight of safety and data integrity in the study. Describe the composition of the SMC or DSMB, frequency of interim data review, final data analysis and method of reviews. A separate DSMB Charter will provide further detail of DSMB membership, responsibilities and administration of the DSMB.

Example text provided as a guide, customize as needed:

[Safety oversight will be under the direction of a Data and Safety Monitoring Board (DSMB) composed of individuals with the appropriate expertise, including <list expertise>. Members of the DSMB should be independent from the study conduct and free of conflict of interest, or measures should be in place to minimize perceived conflict of interest. The DSMB will meet at least semiannually to assess safety and efficacy data on each arm of the study. The DSMB will operate under the rules of an approved charter that will be written and reviewed at the organizational meeting of the DSMB. At this time, each data element that the DSMB needs to assess will be clearly defined. The DSMB will provide its input to <specify the study Sponsor/National Institutes of Health staff/other>.]

10.10 Protocol Violations/Deviations

- Define which deviations from the protocol will be considered a violation of the protocol.
- Describe the ramifications of any defined protocol violations/deviations.

Note: Typically, protocol "violations" include deviations from the inclusion and exclusion criteria, concomitant medication restrictions, and any other protocol requirement that results in a significant added risk to the subject or has an impact on the quality of the data collected or the outcome of the study. A "deviation" occurs when there is non-adherence to study procedures or schedules, as specified by the protocol, which does not involve inclusion/exclusion criteria or the primary endpoint and which does not place the subject at any added risk or affect the data quality or study outcome. Examples of "deviations" may include common out-of-window visits, a missed procedure, etc. Protocol violations must be reported in the final clinical study report, whereas protocol deviations may be mentioned but are not required to be reported.

Plans for detecting, reviewing, and reporting deviations from the protocol should be described. A statement should be included to indicate that deviations are not allowed, unless a statement is included in the Investigator agreement.

Provisions for approval of deviations can be described.

Example text provided as a guide, customize as needed:

[A protocol deviation is any noncompliance with the clinical trial protocol, ICH GCP, or Manual of Procedures (MOP) requirements. The noncompliance may be either on the part of the subject, the Investigator, or the study site staff. As a result of deviations, corrective actions are to be developed by the site and implemented promptly.

These practices are consistent with ICH GCP E6 (R2):

- 4.5 Compliance with Protocol, sections 4.5.1, 4.5.2, and 4.5.3.
- 5.1 Quality Assurance and QC, section 5.1.1.
- 5.20 Noncompliance, sections 5.20.1, and 5.20.2.

It is the responsibility of the site Investigator to use continuous vigilance to identify and report deviations within <specify number> working days of identification of the protocol deviation, or within <specify number> working days of the scheduled protocol-required activity. All deviations must be addressed in study source documents, reported to <specify NIH Institute or Center (IC)> Program Official and <specify DCC or Sponsor>. Protocol deviations must be sent to the reviewing IRB per their policies. The site Investigator is responsible for knowing and adhering to the reviewing IRB requirements. Further details about the handling of protocol deviations will be included in the MOP.]

10.11 Access to Source Documentation

- Define which study subjects will be able to access the source documentation and when.
- Include a statement indicating that the study site must allow trial-related monitoring audits, IRB/IEC review, and regulatory inspection(s), with direct access to source data/documents.

Example text provided as a guide, customize as needed:

[Source documents provide evidence for the existence of the subject and substantiate the integrity of the data collected. Source documents including web-based documentation are to be filed at the study site.

Data reported and entered in the CRF that are transcribed from source documents must be consistent with the source documents or the discrepancies must be explained. The Investigator may need to request previous medical records or transfer records, depending on the study. Also, current medical records must be available for all data collected in the CRF.

For CRFs that are direct data entry, the CRFs will serve as the source document.

Refer to the Monitoring Plan for source data verification review and requirements.

Source documents are subject to inspection by the Sponsor or regulatory agencies including FDA. During audits and inspections, the Investigator must permit direct access to all of the source documentation and other documents pertinent to this study.]

10.12 Data Generation and Analysis

Provide the following information:

- Who will have responsibility for data processing,
- What program will be used to generate the database,
- How data will be entered into the database and the timing of data processing,
- The procedures for management of data clarification and resolution, and
- Any measures to be taken to ensure data quality.

10.13 Retention of Data

Specify the length of time for the Investigator to maintain all records pertaining to this study. The Investigator should use the most conservative rule for document retention – ie, retention should follow the rule that has the longest period.

Indicate whether permission is required (and from whom) prior to destruction of records. If under an IND/IDE, records should not be destroyed without the IND/IDE Sponsor's agreement. Pharmaceutical companies who supply unapproved products should be consulted.

Study intervention records may be described here if not addressed elsewhere in the protocol.

Example text provided as a guide, customize as needed:

[Study documents should be retained for a minimum of <2/25> years after the last approval of a marketing application in an ICH region and until there are no pending or contemplated marketing applications in an ICH region or until at least <2/25> years have elapsed since the formal discontinuation of clinical development of the study intervention. These documents should be retained for a longer period, however, if required by local regulations. No records will be destroyed without the written consent of the Sponsor, if applicable. It is the responsibility of the Sponsor to inform the Investigator when these documents no longer need to be retained.]

10.14 Financial Disclosure

- Describe any requirements regarding financial disclosure by the Investigators or other study personnel.

10.15 Publication and Disclosure Policy

- Describe any publication and disclosure policies for this study.

11 REFERENCE LIST

- Insert a list of references cited in the protocol numerically according to the order presented in the protocol.

1.

References should be cited in accordance with the internationally accepted standards of the 1979 Vancouver Declaration on Uniform Requirements for Manuscripts Submitted to Biomedical Journals or the system used in Chemical Abstracts.

Copies of important publications should be attached in an appendix.

Appendix 1 Schedule of Events

The study events table is intended as an aid for study site personnel and must therefore accurately reflect the type and timing of all study events.

The following table is provided as a sample study events table. Modify the content and add or delete rows as applicable for the protocol.

With studies that require frequent measurements, the Study Events Table may be best displayed on a landscaped page.

			Treatment Period						Follow-up
Evaluation	**Screening Days -x to -1**	**Visit 1; Day 1**	**Visit 2; Day x ± x**	**Visit 3; Day x ± x**	**Visit 4; Day x ± x**	**Visit 5; Day x ± x**	**Visit 6; Day x ± x**	**Visit 7; Day x ± x**	**Visit 8; Day x ± x**
Informed Consent	X								
Inclusion/Exclusion Criteria	X								
Demographics	X								
Medical History[1]	X	X							
Physical Examination	X[2,3]	X[4]							X[2]
Urine Drug Screen[5]	X	X							
Pregnancy Test[6]	X	X							
Laboratory Tests[7]	X				X				X
Randomization		X							
Vital Signs[8]	X	X			X	X		X	X
12-Lead ECG	X	X	X	X	X	X		X	X
Drug Administration		X	X	X	X	X	X		
Drug Accountability								X	
AE Assessment		X	X	X	X	X	X	X	X

Conmed Assessment		X	X	X	X	X	X	X	X

1. Include a review of previous/ongoing medications.
2. Complete examination, including assessments of the skin, head, eyes, ears, nose, throat, neck, thyroid, lungs, heart, abdomen, lymph nodes, extremities, and body weight.
3. Height will be measured at screening only.
4. Partial examination, to update findings from the examination performed at screening.
5. Includes amphetamines, barbiturates, cocaine metabolites, opiates, benzodiazipines, cannabinoids, and cotinine.
6. Only for female subjects who are not diagnosed as postmenopausal.
7. Include serum chemistry, hematology (including coagulation), and urinalysis.
8. Include blood pressure, pulse, respiratory rate, and oral temperature.

Appendix 2 Study Drug Preparation and Dispensing Instructions

Appendix 3 Adverse Events: Definitions and Procedures for Recording, Evaluating, Follow-Up, and Reporting

Use this appendix for drug treatment studies, for studies with medical devices, use the medical device template. For studies with both a drug and device component, include the medical device AEs appendix for regulatory reporting requirements IN ADDITION to the drug AE appendix.

Adverse Event Definition

Provide the definition of an AE being used for the clinical trial. Example text provided as a guide, customize as needed:

An AE is any untoward medical occurrence in a subject administered a pharmaceutical product, irrespective of the whether the event is considered treatment related. An AE can therefore be any unfavorable and unintended sign (including an abnormal laboratory finding), symptom, or disease concurrent with the use of the study drug, whether or not related to the study. This includes any newly occurring event, or a pre-existing condition judged by the Investigator to have increased in severity or frequency since the administration of study drug.

Severity Assessment of Adverse Events

All AEs will be assessed by the study clinician using a protocol-defined grading system. Describe the method of grading an AE for severity. For example, many toxicity tables are available for use and are adaptable to various study designs. Selection of a toxicity table or severity scale should be made in consultation with the study Medical Monitor.

Define AE severity (intensity), eg, mild, moderate, severe.

Example text provided as a guide, customize as needed:

[For AEs not included in the protocol-defined grading system, the following guidelines will be used to describe severity:

- **Mild** – Events require minimal or no treatment and do not interfere with the subject's daily activities.
- **Moderate** – Events result in a low level of inconvenience or concern with the therapeutic measures. Moderate events may cause some interference with functioning.
- **Severe** – Events interrupt a subject's usual daily activity and may require systemic drug therapy or other treatment. Severe events are usually potentially life-threatening or incapacitating. Of note, the term "severe" does not necessarily equate to "serious".]

Relationship of Adverse Events to Study Treatment

All AEs will have their relationship to study intervention or study participation assessed with a level of specificity appropriate to the study design. The clinician's assessment of an AE's relationship to study intervention (drug, biologic, device) is part of the documentation process, but it is not a factor in determining what is or is not reported in the study. Describe the method of determining the relationship of an AE to a study intervention. If there is any doubt as to whether a clinical observation is an AE, the event should be reported. Some protocols may use a binary assessment (related/not related); others may have a scale of relatedness. Evaluation of relatedness must consider etiologies such as natural history of the underlying disease, concurrent illness, concomitant therapy, study-related procedures, accidents, and other external factors. In a clinical trial, the study intervention must always be suspect.

- Define any relationship the AE may have to the study drug or any comparator, eg, not related, possibly related, probably related, or definitely related.
- Identify the person responsible for determining the AE relationship (usually the Investigator).
- Define how to determine the relationship of AEs to the study drug, route of administration, dosage, dosage form or duration of therapy, if applicable.

Example text provided as a guide, customize as needed:

[All AEs must have their relationship to study intervention assessed by the clinician who examines and evaluates the subject based on temporal relationship and his/her clinical judgment. The degree of certainty about causality will be graded using the categories below. In a clinical trial, the study product must always be suspect.

- **Related** – The AE is known to occur with the study intervention, there is a reasonable possibility that the study intervention caused the AE, or there is a temporal relationship between the study intervention and event. Reasonable possibility means that there is evidence to suggest a causal relationship between the study intervention and the AE.
- **Not Related** – There is not a reasonable possibility that the administration of the study intervention caused the event, there is no temporal relationship between the study intervention and event onset, or an alternate etiology has been established.

OR

- **Definitely Related** – There is clear evidence to suggest a causal relationship, and other possible contributing factors can be ruled out. The clinical event, including an abnormal laboratory test result, occurs in a plausible time relationship to study intervention administration and cannot be explained by concurrent disease or other drugs or chemicals. The response to withdrawal of the study intervention (dechallenge) should be clinically plausible. The event must be pharmacologically or phenomenologically definitive, with use of a satisfactory rechallenge procedure if necessary.

- **Probably Related** – There is evidence to suggest a causal relationship, and the influence of other factors is unlikely. The clinical event, including an abnormal laboratory test result, occurs within a reasonable time after administration of the study intervention, is unlikely to be attributed to concurrent disease or other drugs or chemicals, and follows a clinically reasonable response on withdrawal (dechallenge). Rechallenge information is not required to fulfill this definition.
- **Possibly Related** – There is some evidence to suggest a causal relationship (eg, the event occurred within a reasonable time after administration of the trial medication). However, other factors may have contributed to the event (eg, the subject's clinical condition, other concomitant events). Although an AE may rate only as "possibly related" soon after discovery, it can be flagged as requiring more information and later be upgraded to "probably related" or "definitely related", as appropriate.
- **Unlikely to be related** – A clinical event, including an abnormal laboratory test result, whose temporal relationship to study intervention administration makes a causal relationship improbable (eg, the event did not occur within a reasonable time after administration of the study intervention) and in which other drugs or chemicals or underlying disease provides plausible explanations (eg, the subject's clinical condition, other concomitant treatments).
- **Not Related** – The AE is completely independent of study intervention administration, and/or evidence exists that the event is definitely related to another etiology. There must be an alternative, definitive etiology documented by the clinician.]

Expectedness of Adverse Events

Expected adverse reactions are AEs that are known to occur for the study intervention being studied and should be collected in a standard, systematic format using a grading scale based on functional assessment or magnitude of reaction. Identify the source of the reference safety information used to determine the expectedness of the AE (eg, IB, approved labeling). Expectedness is assessed based on the awareness of AEs previously observed, not on the basis of what might be anticipated from the properties of the study intervention.

An AE or suspected adverse reaction is considered "unexpected" if it is not listed in the IB, package insert, or device labeling or is not listed at the specificity or severity that has been observed; or, if an IB is not required or available, is not consistent with the risk information described in the protocol, as amended. For example, under this definition, hepatic necrosis would be unexpected (by virtue of greater severity) if the IB or package insert referred only to elevated hepatic enzymes or hepatitis. Similarly, cerebral thromboembolism and cerebral vasculitis would be unexpected (by virtue of greater specificity) if the IB or package insert listed only cerebral vascular accidents. "Unexpected," as used in this definition, also refers to AEs or suspected adverse reactions that are mentioned in the IB, package insert, or device labeling as occurring with a class of drugs (or other medical products) or as anticipated from the pharmacological properties or other characteristics of the study intervention, but

are not specifically mentioned as occurring with the particular study intervention under investigation.

Example text provided as a guide, customize as needed:

[<Insert role> will be responsible for determining whether an AE is expected or unexpected. An AE will be considered unexpected if the nature, severity, or frequency of the event is not consistent with the risk information previously described for the study intervention.]

- Define any AEs that are expected, based on the IB and previous clinical trials.
- Describe the means for determining and reporting unexpected AEs.

Treatment-Emergent Adverse Events

Provide the definition of a TEAE here.

[TEAEs are defined as an event that emerges during treatment, having been absent pretreatment, or worsens relative to the pretreatment state.]

Serious Adverse Event Definition

Provide the definition of an SAE being used for the clinical trial. Example text provided as a guide, customize as needed:

[An SAE is defined by federal regulation as any AE occurring at any dose that results in any of the following outcomes: death, life-threatening AE, hospitalization or prolongation of existing hospitalization, a persistent or significant disability/incapacity, or a congenital anomaly/birth defect.

Important medical events that may not result in death, be life-threatening, or require hospitalization may be considered an SAE when, based upon appropriate medical judgment, they may jeopardize the subject and may require medical or surgical intervention to prevent one of the outcomes listed in this definition. Examples of such medical events include allergic bronchospasm requiring intensive treatment in an emergency room or at home, blood dyscrasias or convulsions that do not result in inpatient hospitalization, or the development of drug dependency or drug abuse.]

Serious Adverse Event Follow-Up

Define how SAEs will be followed for their duration. Suggested text below, modify as needed for the study.

[The Investigator must respond to all queries received from the Sponsor and must submit follow-up reports for all SAEs to the Sponsor or Sponsor's designee regarding the subject's subsequent course. All SAEs must be followed until:

- The SAE is resolved, or
- Stabilized (eg, in the case of persistent impairment), or
- Subject death, or
- Subject is lost to follow-up.

SAE follow-up information should be recorded in the source documents and SAE/CRF forms as applicable.]

Appendix 4 Prohibited Concomitant Medications

Consider including a chart of prohibited concomitant medications as an appendix, especially if the list is long and if there is a potential for error.

Sample text follows:

The following medications are prohibited during the study:

Trade Name **Generic Name**	**Trade Name(s)**
Antibiotics	

1

Appendix 5 Contraceptive and Barrier Guidance

Delete this appendix if not applicable to the study.

Definitions

Woman of Childbearing Potential (WOCBP)

Women in the following categories are considered WOCBP (fertile):

1. Following menarche
2. From the time of menarche until becoming postmenopausal unless permanently sterile (see below)

- A postmenopausal state is defined as no menses for 12 months without an alternative medical cause.
  - A high follicle-stimulating hormone (FSH) level in the postmenopausal range may be used to confirm a postmenopausal state in women not using hormonal contraception or hormonal replacement therapy (HRT). However, in the absence of 12 months of amenorrhea, confirmation with more than one FSH measurement [insert threshold if required (> 40 IU/L or mIU/mL) or remove to allow for flexibility with different local thresholds for defining postmenopausal state] is required.
  - Female subjects on HRT and whose menopausal status is in doubt will be required to use one of the nonestrogen hormonal highly effective contraception methods if they wish to continue their HRT during the study. Otherwise, they must discontinue HRT to allow confirmation of postmenopausal status before study enrollment.
- Permanent sterilization methods (for the purpose of this study) include:
  - Documented hysterectomy.
  - Documented bilateral salpingectomy.
  - Documented bilateral oophorectomy.
  - For individuals with permanent infertility due to an alternate medical cause other than the above, (eg, Mullerian agenesis, androgen insensitivity, gonadal dysgenesis), Investigator discretion should be applied to determining study entry.

Note: Documentation can come from the site personnel's review of the subject's medical records, medical examination, or medical history interview.

- If fertility is unclear (eg, amenorrhea in adolescents or athletes) and a menstrual cycle cannot be confirmed before the first dose of study intervention, additional evaluation should be considered.

Contraceptive Guidance

Select relevant wording to include in protocol from options below. For studies enrolling only male subject, the appropriate text option aligned to female subject risk should be included when appropriate.

All Female Subject Options: If a clinically relevant interaction between study intervention(s) and contraceptive steroids has been observed or is suspected that may compromise efficacy of hormonal contraception, hormonal contraception should not be used. If the interaction observed however is not considered to exert a clinically relevant compromise to the efficacy of hormonal contraception, the hormonal contraception method should be supplemented with male condom.

Table 2 Contraceptives Allowed During Study Participation

CONTRACEPTIVES[a] ALLOWED DURING THE STUDY INCLUDE:
Highly Effective Methods[b] That Have Low User Dependency
● Implantable progestogen-only hormone contraception associated with inhibition of ovulation.
● Intrauterine device.
● Intrauterine hormone-releasing system[c].
● Bilateral tubal occlusion.
● Azoospermic partner (vasectomized or due to a medical cause). *Azoospermia is a highly effective contraceptive method provided that the partner is the sole sexual partner of the WOCBP and the absence of sperm has been confirmed. If not, an additional highly effective method of contraception should be used. Spermatogenesis cycle is approximately 90 days.* Note: documentation of azoospermia for a male subject can come from the site personnel's review of the subject's medical records, medical examination, or medical history interview.
Highly Effective Methods[b] That Are User Dependent
● Combined (estrogen- and progestogen-containing) hormonal contraception associated with inhibition of ovulation: o Oral. o Intravaginal. o Transdermal. o Injectable.
● Progestogen-only hormone contraception associated with inhibition of ovulation[c]: o Oral. o Injectable.
● Sexual abstinence. *Sexual abstinence is considered a highly effective method only if defined as refraining from heterosexual intercourse during the entire period of risk associated with the study intervention. The reliability of sexual abstinence needs to be evaluated in relation to the duration of the study and the preferred and usual lifestyle of the subject.*
a. Contraceptive use by men or women should be consistent with local regulations regarding the use of contraceptive methods for those participating in clinical studies. b. Failure rate of < 1% per year when used consistently and correctly. Typical use failure rates differ from those when used consistently and correctly.

c. [Male condoms must be used in addition to hormonal contraception]. If locally required, in accordance with Clinical Trial Facilitation Group (CTFG) guidelines, acceptable contraceptive methods are limited to those which inhibit ovulation as the primary mode of action.

Note: Periodic abstinence (calendar, symptothermal, postovulation methods), withdrawal (coitus interruptus), spermicides only, and lactational amenorrhea method are not acceptable methods of contraception for this study. Male condom and female condom should not be used together (due to risk of failure from friction).

Appendix 6 European Union-Specific Regulatory Considerations

Delete this appendix if no clinical sites are planned to be opened in the EU.

Requirements for a clinical trial protocol for a study that will take place in the EU are laid out in Annex D of the EU Clinical Trials Regulation (CTR), which can be consulted here: https://emmes.sharepoint.com/:b:/r/sites/EmmesMedicalWriting/Shared%20Documents/General/Guidance%20Documents/EU%20Clinical%20Trial%20Regulation%20536-2014.pdf?csf=1&web=1&e=LMHVB8

The Medical Writer should ensure that the protocol contains information concerning all of the items specified in Annex D of the CTR. If any information required by the EU CTR is missing from the protocol, add an appropriate subsection to this appendix and add the information there.

The following information is mandatory in protocols of clinical trials conducted in any EU member state:

General Considerations

The study will be conducted in accordance with the protocol, European Union (EU) Regulation 536/2014 (Clinical Trials Regulation [CTR]) and Good Clinical Practice (GCP).

EU Trial Number

The study has been registered in the EU under EU Trial Number .

Subject Data Protection

In accordance with GCP and with national data protection laws, all information concerning the subjects in the clinical trial must be treated as strictly confidential by all persons involved in the clinical trial. All activities related to EU citizen data will be conducted in accordance with the applicable requirements of the EU 2016/679 General Data Protection Regulation (GDPR) and regional data protection requirements. Data collected in the clinical trial will be recorded and forwarded to the Sponsor or its designee as per applicable law. The site shall grant access to subjects´ data to participating IECs, authorized sponsor representatives (such as monitors and auditors) and regulatory authorities, for auditing and regulatory purposes as required by applicable law.

The Investigator and the Sponsor will ensure that the collection, use, storage and disclosure of protected health and personal information obtained during a clinical trial complies with all applicable EU Member State privacy laws and regulations, including but not limited to the EU GDPR. Data protection in accordance with EU GDPR is applicable even if EU citizen data are transferred to other countries outside EU. Authorization is required from each research subject (i.e., specific permission granted by an individual for the use or disclosure of an individual's protected personal or health information) as per applicable law.

Appendix 7 Sponsor Signatures

Study Title: {study title}

Study Number: {study number}

Final Date: **Error! Reference source not found.**

This clinical study protocol was subject to critical review and has been approved by the Sponsor. The following personnel contributed to writing and/or approving this protocol:

Signed:_____ Date:_____

Signed:_____ Date:_____

Signed:_____ Date:_____

Appendix 8 Investigator's Signature

Study Title: {study title}

Study Number: {study number}

Final Date: **Error! Reference source not found.**

The following sample text is provided.

I have read the protocol described above. I agree to comply with all applicable regulations and to conduct the study as described in the protocol.

Signed:_____ Date:_____